PRACTICE EXAM 33 / EXAM SET 05

Domain 5: Administrative Assisting

12 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Scheduling and Patient Flow

> **SCENARIO** A provider instructs the scheduling team to arrange a laboratory specimen collection before the patient's office visit on the same morning; results are not required before the visit. The office visit is confirmed for 9:00 a.m., but the laboratory appointment is at 10:00 a.m. The laboratory can offer 8:30 a.m., and the patient confirms that this time works. Both departments are in the same office, and the collection will finish before 9:00 a.m.

Which scheduling change follows the provider's instruction?

A. Keep both bookings because they occur on the same morning.
B. Move the office visit to 8:00 a.m. and keep the 10:00 a.m. collection.
C. Move the collection to 8:30 a.m. and retain the 9:00 a.m. office visit.
D. Cancel the collection because results will not be ready for the office visit.

Q02 | Coding Billing and Payments

> **SCENARIO** During charge review, the assistant compares a proposed procedure entry with the signed encounter documentation. The provider documents treatment of the left knee, while the proposed procedure description specifies the right knee. The patient identifiers, date of service, and rendering provider all match. The coding team corrects procedure entries before release.

Which detail should the assistant flag for correction?

A. The date of service associated with the encounter.
B. The treated side in the proposed procedure entry.
C. The patient identity associated with the charge.
D. The rendering provider listed for the service.

Q03 | Records Office Systems and Supplies

SCENARIO The purchasing lead asks the assistant to compare two approved suppliers of identical label rolls. Vendor A offers 12 rolls for $36 plus $12 shipping. Vendor B offers 20 rolls for $50 plus $10 shipping. There are no taxes, discounts, or other fees, and all rolls will be usable. The purchasing decision is based on delivered cost per roll.

Which option has the lower delivered cost per roll?

A. Vendor A at $3 per roll.
B. Vendor A at $4 per roll.
C. Vendor B at $2.50 per roll.
D. Vendor B at $3 per roll.

Q04 | Insurance and Prior Authorization

SCENARIO An automated checklist flags a missing authorization for a routine office follow-up. Current eligibility and the visit benefit have been verified. The payer's written response explicitly states that this visit type requires no prior authorization. Office policy allows staff to close an inapplicable checklist alert by recording the payer verification and its reference number.

Which action resolves the checklist discrepancy?

A. Document that authorization is not required, retain the verification reference, and close the inapplicable alert.
B. Record the visit as authorization-approved even though no authorization was requested.
C. Cancel the visit until the payer issues an authorization number.
D. Submit an authorization request and leave the visit on hold despite the verified exemption.

Q05 | Records Office Systems and Supplies

SCENARIO A returning patient now uses a different legal last name. The patient supplies the former name and date of birth, and the office's verification process confirms that an existing record under the former name belongs to this patient. Policy directs staff to update the verified name on the existing record and retain the former name as a search alias.

Which registration action follows the policy?

A. Create a second permanent patient record under the new last name.
B. Replace the date of birth so a new record will not resemble the existing record.
C. Use the verified existing record, update the name, and retain the former name as an alias.
D. Register the visit without a medical record number until the former chart is deleted.

Q06 | Coding Billing and Payments

SCENARIO The billing team confirms a $70 patient overpayment after all claims for the encounter are finalized. There are no other balances or pending claims on the account. The patient requests a refund. Office policy requires supervisor approval and a documented refund transaction linked to the original payment; the assistant may prepare the request but may not issue refunds independently.

Which action should the assistant take?

A. Prepare the supported refund request for supervisor approval and documented processing.
B. Delete the original payment so the credit balance no longer appears.
C. Keep the overpayment for a future visit despite the patient's refund request.
D. Give the patient cash from the drawer without recording a refund transaction.

Q07 | Scheduling and Patient Flow

SCENARIO A patient without an appointment asks for a routine visit today. The clinical nurse has reviewed the request and confirmed that routine scheduling is appropriate. Every regular appointment is filled. Clinic policy permits overbooking only after the scheduling coordinator approves it; the coordinator is available.

Which action is within the assistant's scheduling role?

A. Add the patient to the next occupied slot without asking the coordinator.
B. Reclassify the visit as urgent to create an exception to the scheduling rule.
C. Tell the patient the provider has approved an extra visit because the nurse reviewed the request.
D. Ask the coordinator about an approved overbook or another authorized scheduling option.

Q08 | Insurance and Prior Authorization

SCENARIO A payer denies a nonurgent test authorization for lack of medical necessity. The provider wants to appeal and will supply the clinical rationale. The denial notice identifies the appeal route, deadline, and required documents. Office policy assigns the assistant to organize the packet and track its status, while the provider is responsible for the clinical argument.

Which task should the assistant perform?

A. Write a new diagnosis that is more likely to satisfy the payer.
B. Track the appeal deadline and assemble the required documents with the provider's rationale.
C. Choose a replacement test and tell the patient to follow that plan.
D. Treat the denied request as approved once the provider disagrees with the decision.

Q09 | Scheduling and Patient Flow

SCENARIO The manager is building a clinic session from 1:00 to 4:00 p.m. The session must include a 20-minute team huddle and 40 minutes of non-visit administrative time. Each routine appointment requires 15 minutes. Visits cannot overlap, the reserved non-visit time cannot be used for appointments, and only complete appointments may be booked.

What is the maximum number of routine appointments that fit in the session?

A. 12 appointments.
B. 10 appointments.
C. 9 appointments.
D. 8 appointments.

Q10 | Insurance and Prior Authorization

SCENARIO The provider documents that today's visit concerns a reported workplace injury. Office policy routes these accounts to a workers' compensation billing coordinator to verify the applicable billing pathway. The assistant has been asked to collect available administrative claim details, without deciding whether the injury is compensable.

Which information set is most useful for that coordinator?

A. The employer, reported injury date, workers' compensation carrier or administrator, and claim number if assigned.
B. The patient's preferred payment method, billing address, and ordinary statement delivery preference.
C. The patient's preferred specialist, referral preferences, and ordinary health-plan network list.
D. The regular health-plan member ID, group number, and routine office-visit copayment.

Q11 | Coding Billing and Payments

When paper submission is permitted, which standard claim form is used for professional services furnished by a physician's office?

A. CMS-1500.
B. CMS-1450 (UB-04).
C. A patient billing statement.
D. An explanation of benefits.

Q12 | Records Office Systems and Supplies

SCENARIO The manager approves version 3 of the blank patient registration form for use starting Monday. Version 2 templates and unused printed copies are still available at check-in. Office policy requires current blank forms to replace obsolete blanks, while completed forms already filed in patient records remain part of those records.

Which action implements the approved version change?

A. Keep both blank versions available so staff can choose whichever is easier to find.
B. Remove completed version 2 forms from patient records along with the unused copies.
C. Replace obsolete blank templates and unused stock with version 3 while retaining completed record forms.
D. Change only the version number printed on existing version 2 blanks without replacing their contents.

ANSWER KEY & EXPLANATIONS

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01 C	02 B	03 D	04 A	05 C	06 A
07 D	08 B	09 D	10 A	11 A	12 C

DETAILED EXPLANATIONS

Q01 | Correct answer: C

Move the collection to 8:30 a.m. and retain the 9:00 a.m. office visit.

The ordered sequence is specimen collection followed by the office visit, and the offered 8:30 a.m. collection meets that sequence without changing the confirmed visit. The stem specifically states that results are not needed beforehand. Having both appointments on the same morning does not correct their reversed order.

Q02 | Correct answer: B

The treated side in the proposed procedure entry.

The documented left side conflicts with the right side in the proposed entry. The assistant should flag that laterality discrepancy for the coding team rather than release a mismatched entry. The other listed details have already been verified as matching, and no clinical diagnosis needs to be inferred.

Q03 | Correct answer: D

Vendor B at $3 per roll.

Vendor A costs ($36 + $12) / 12 rolls = $4 per roll delivered. Vendor B costs ($50 + $10) / 20 rolls = $3 per roll delivered, which is lower. The $3 figure for Vendor A and $2.50 figure for Vendor B omit shipping and therefore are not delivered unit costs.

Q04 | Correct answer: A

Document that authorization is not required, retain the verification reference, and close the inapplicable alert.

The verified payer instruction establishes that authorization is not required for this visit type. The assistant should document that finding through the permitted checklist process. Not required is a different status from approved, and requesting an unnecessary approval would not improve the accuracy of the record.

Q05 | Correct answer: C

Use the verified existing record, update the name, and retain the former name as an alias.

The existing chart has been verified as belonging to the returning patient, so updating that record maintains continuity without creating a duplicate. Retaining the former name as an alias supports future searches. A new surname does not by itself require a new permanent record or a change to other accurate identifiers.

Q06 | Correct answer: A

Prepare the supported refund request for supervisor approval and documented processing.

The verified overpayment and completed claim processing support a refund request through the stated approval process. The original payment must remain part of the financial history, with any approved refund recorded as its own linked transaction. The assistant should not independently issue cash or remove a real payment from the ledger.

Q07 | Correct answer: D

Ask the coordinator about an approved overbook or another authorized scheduling option.

The clinical review establishes the routine pathway, but the stated policy still requires coordinator approval for an overbook. The assistant should obtain that scheduling decision or another authorized option. Clinical review does not automatically authorize extra capacity, and the assistant must not change urgency simply to bypass a booking rule.

Q08 | Correct answer: B

Track the appeal deadline and assemble the required documents with the provider's rationale.

The assistant can organize the administrative appeal, track its deadline, and include the provider's clinical rationale. The supplied policy keeps clinical justification and treatment decisions with the provider. Disagreement with a denial does not itself change the payer's decision or authorize the assistant to alter a diagnosis.

Q09 | Correct answer: D

8 appointments.

The three-hour session contains 180 minutes. Reserving 20 + 40 = 60 minutes leaves 120 minutes for visits, and 120 / 15 = 8 complete appointments. Twelve ignores both reservations; ten and nine result from ignoring one of the reserved blocks and taking only complete visits.

Q10 | Correct answer: A

The employer, reported injury date, workers' compensation carrier or administrator, and claim number if assigned.

Employer, injury-date, carrier or administrator, and claim information help the designated coordinator verify how the reported work-related encounter should be billed. The assistant collects and routes the information under office policy without deciding legal eligibility or guaranteeing payment. An ordinary health-plan copayment or unrelated office record does not establish this billing pathway.

Q11 | Correct answer: A

CMS-1500.

The CMS-1500 is the standard paper professional claim form used when paper submission is permitted. The CMS-1450, also called the UB-04, is an institutional claim form. A patient statement requests payment from a patient, and an explanation of benefits describes payer claim processing rather than serving as the provider's claim form.

Q12 | Correct answer: C

Replace obsolete blank templates and unused stock with version 3 while retaining completed record forms.

The policy distinguishes blank materials used for future registration from completed documents already in the record. Replacing obsolete templates and unused stock implements the new version while preserving existing records. Leaving both blank versions in circulation would permit continued use of the superseded form.

End of Practice Exam